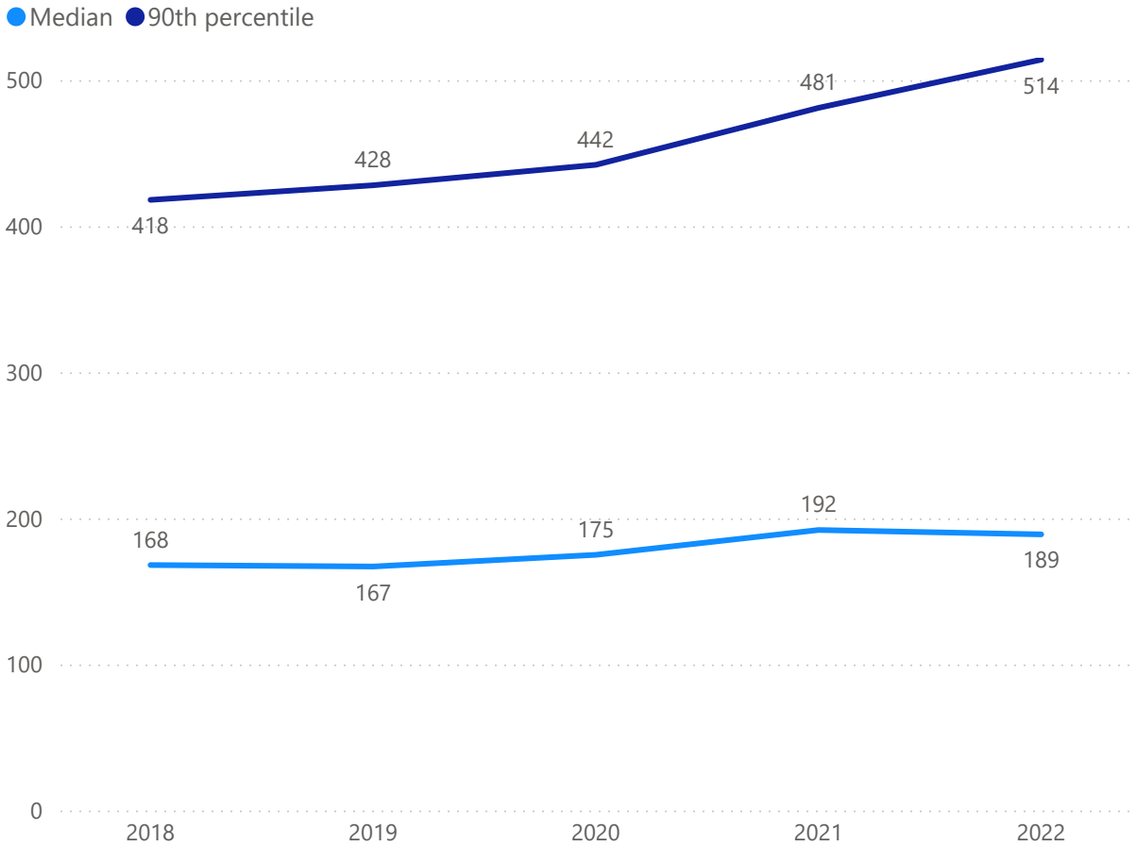
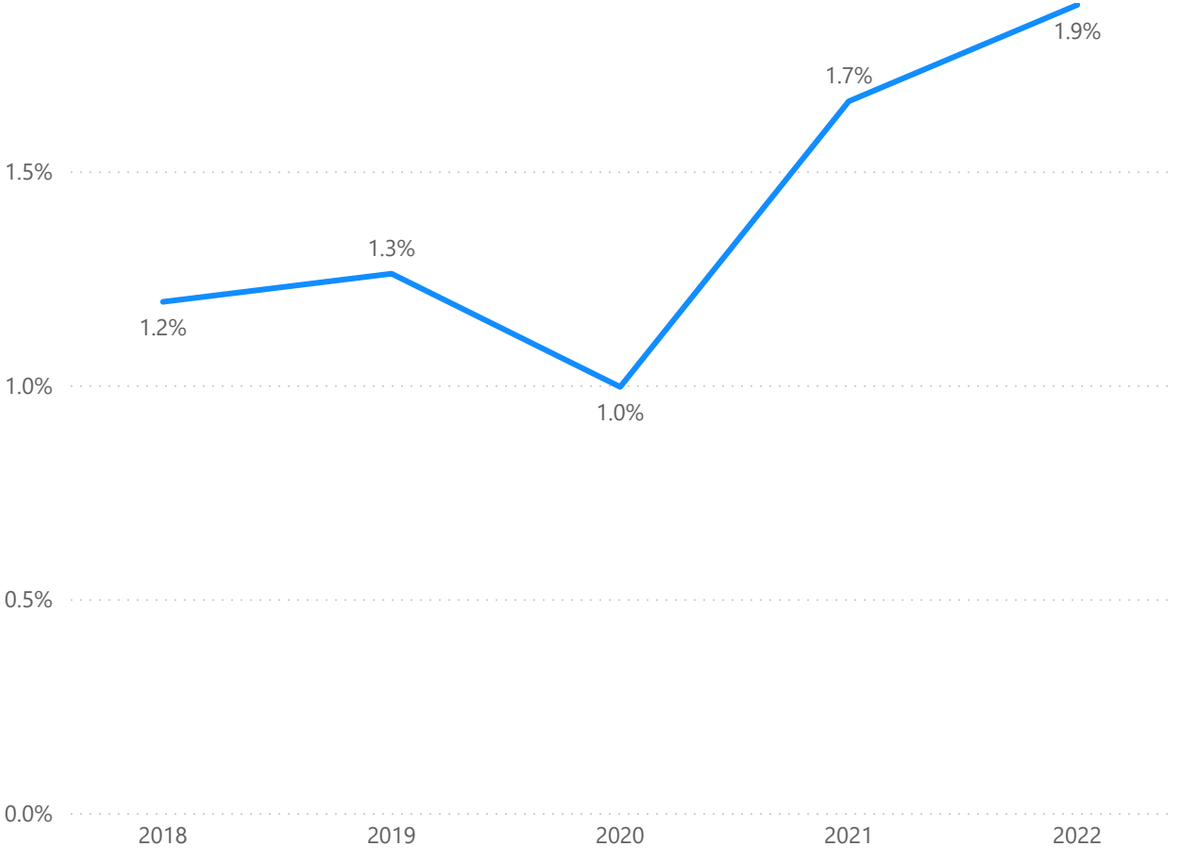
The longest ED visits grew fastest: P90 visit length up 23% from 2018 to 2022

155.4	189	514	23.0%	1.9%
Visits in 2022 (millions)	Median visit, 2022 (min)	P90 visit, 2022 (min)	P90 change since 2018	Left without being seen, 2022

Median and 90th percentile visit length (minutes)



Left without being seen (% of visits)



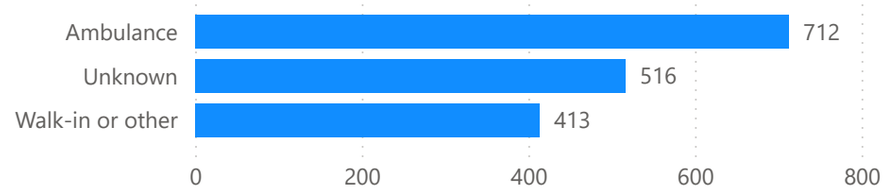
Source: CDC NHAMCS emergency department files, 2018–2022. Weighted national estimates (US). 2020 = COVID-19 year.

Ambulance arrivals and older patients have the longest visits

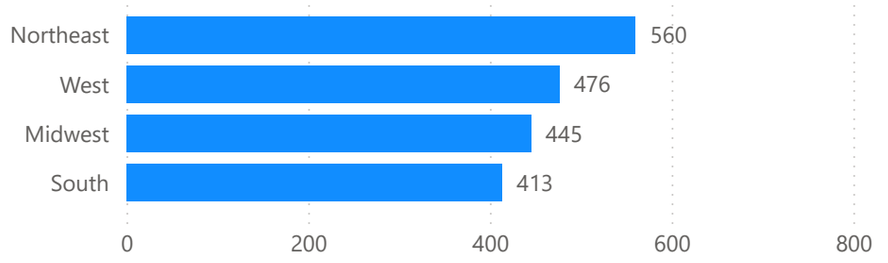
Year
All

P90 visit length in minutes: 1 in 10 visits lasted longer than this

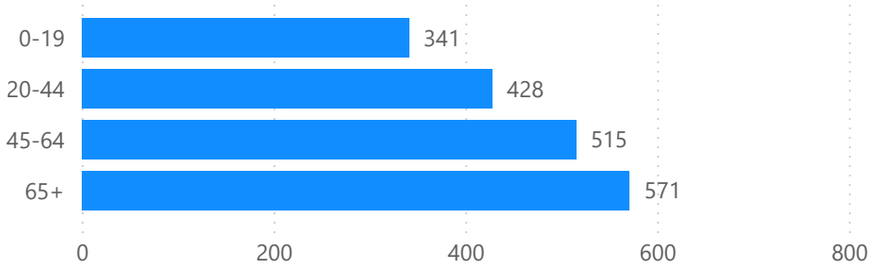
By arrival mode



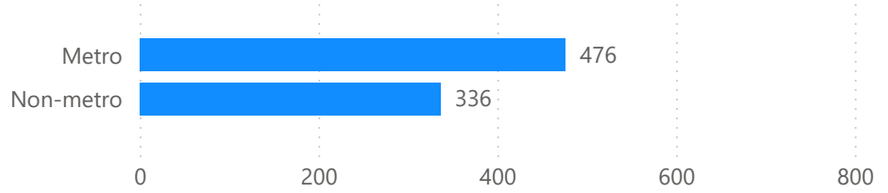
By region



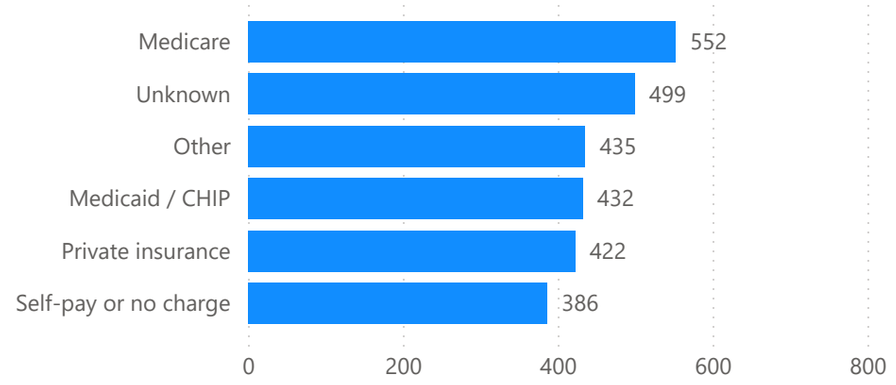
By age group



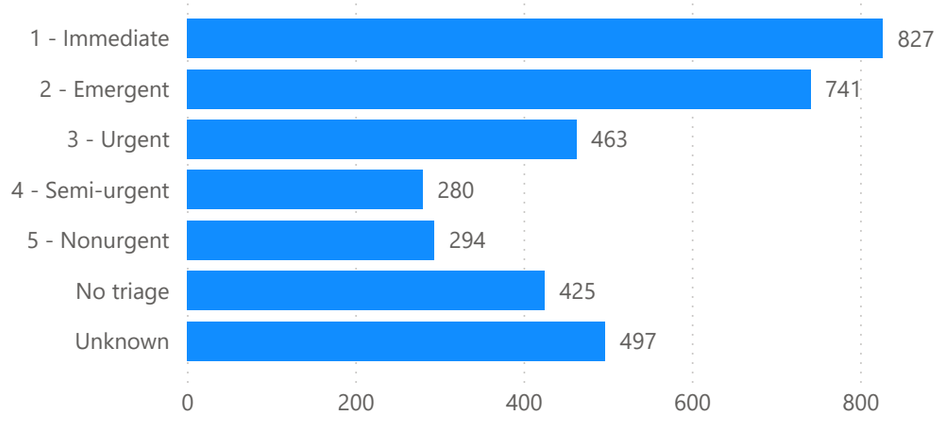
By metro status



By payer group

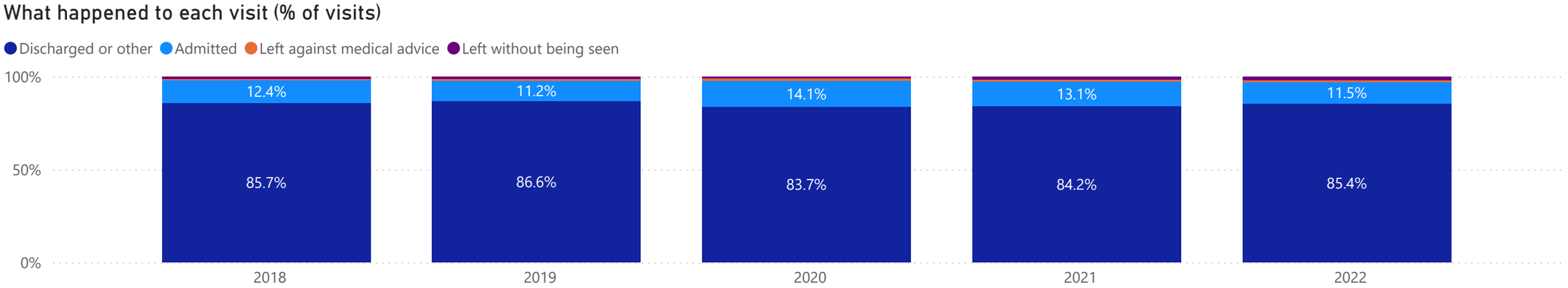


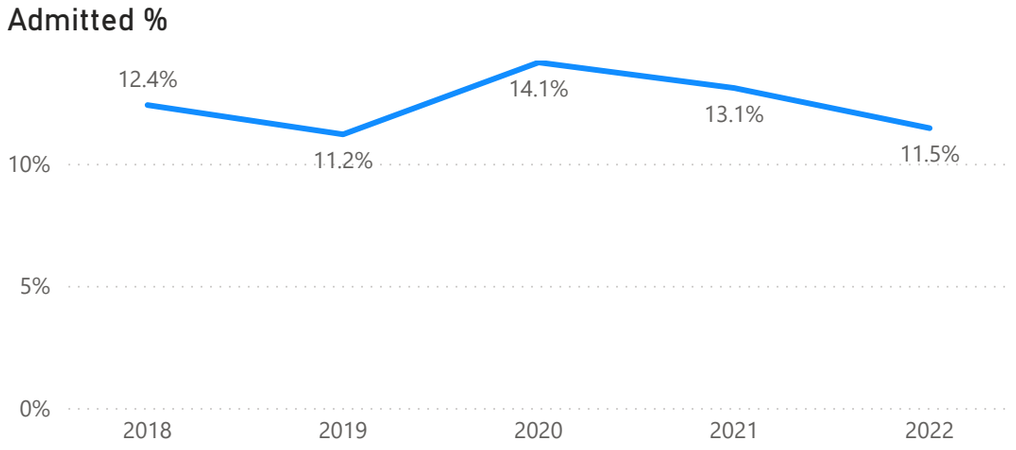
By triage group

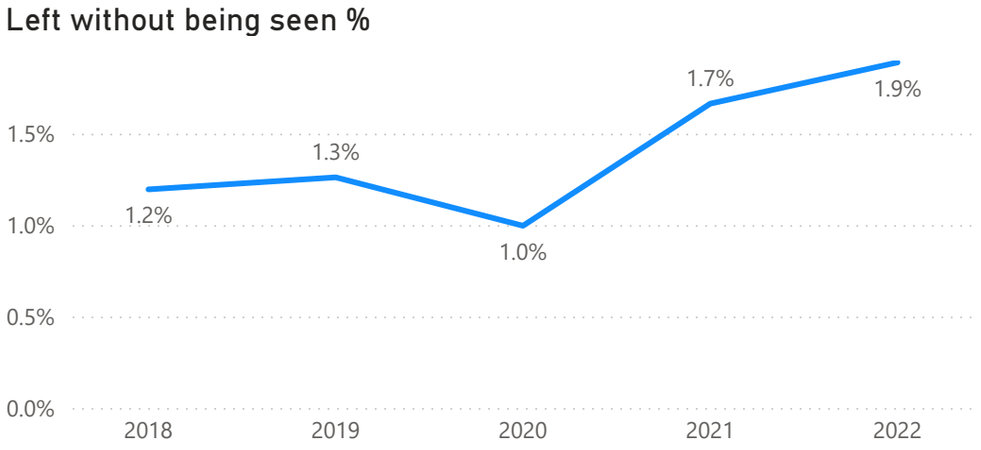


Source: CDC NHAMCS emergency department files, 2018–2022. Weighted national estimates (US). 2020 = COVID-19 year.

About 1 in 8 visits ends in admission; leaving without being seen rose from 1.2% to 1.9%







2020 was the first COVID-19 year.

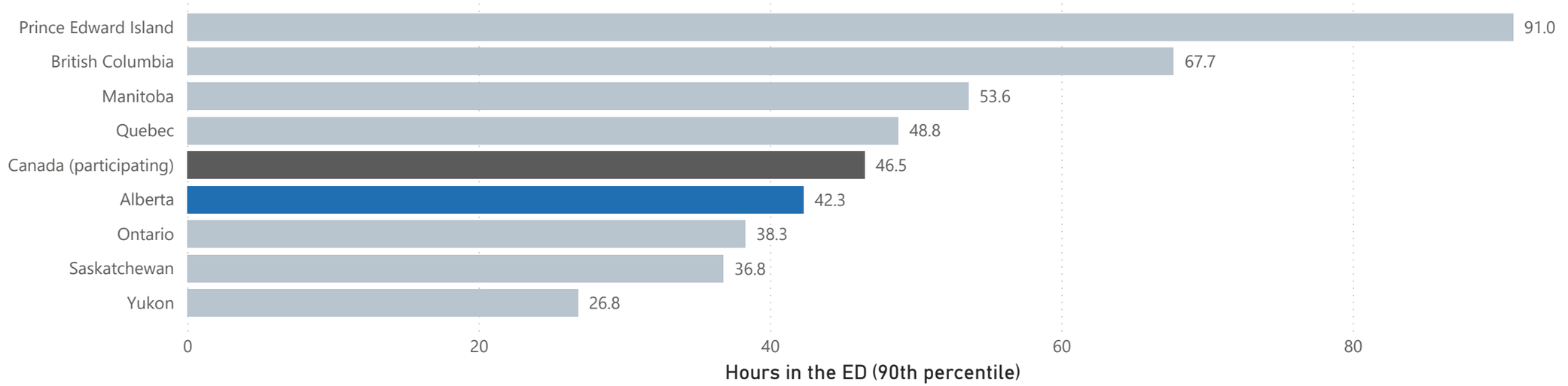
Source: CDC NHAMCS emergency department files, 2018–2022. Weighted national estimates (US). 2020 = COVID-19 year.

Alberta is at or below the Canadian average in every patient group (2025-2026)

Patient group

Admitted

Longest 1 in 10 stays, by province (hours)



42.3
Alberta (hours)

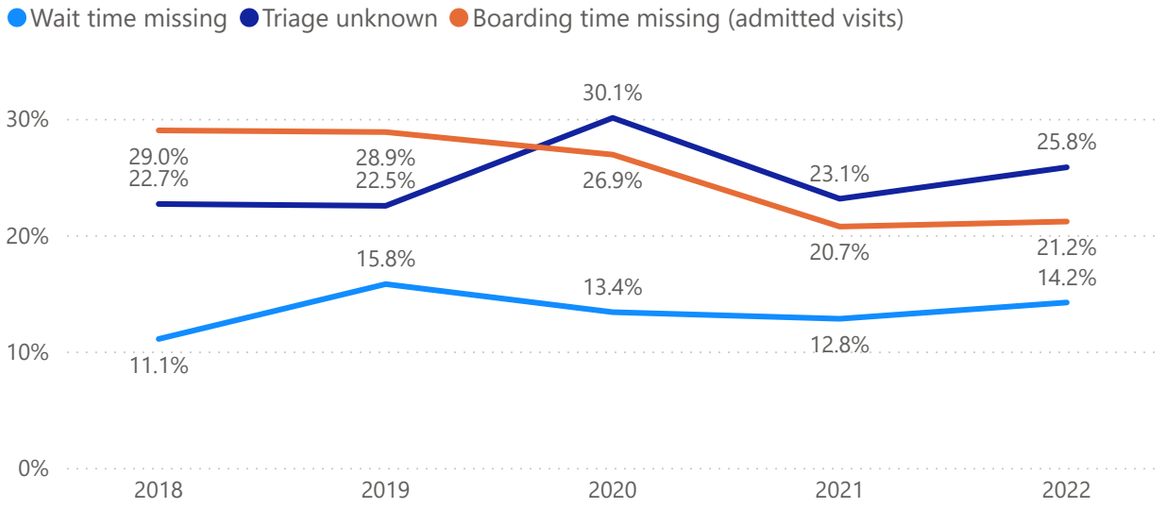
46.5
Canada (hours)

-4.2
Alberta minus Canada (hours)

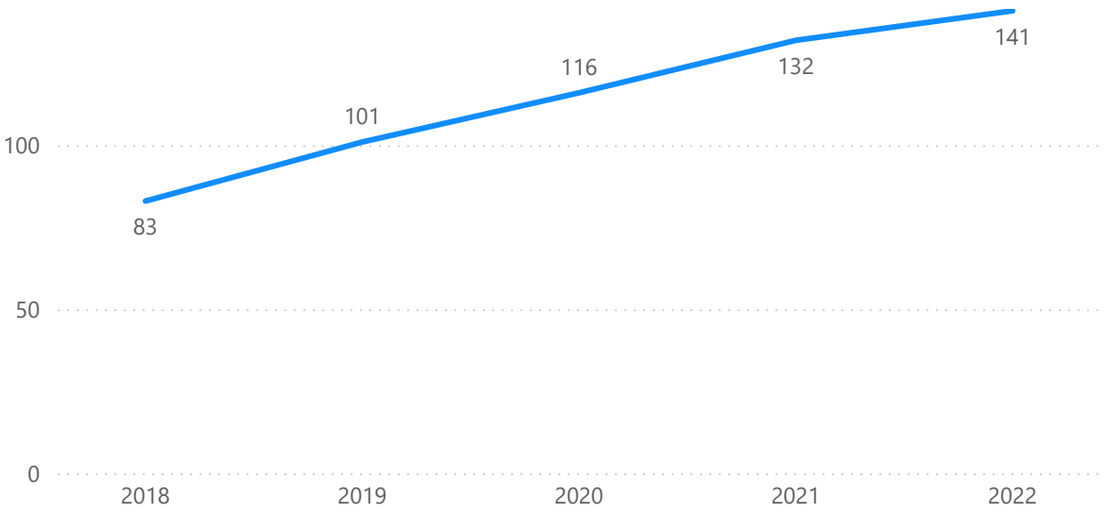
Source: CIHI, NACRS 2025–2026. Hours in the ED. Only some EDs reported in P.E.I., Manitoba, Saskatchewan and B.C.; compare those provinces with care. Nova Scotia: not available.

About 1 in 4 visits has no recorded triage level; some figures need care

Share of visits with missing data (%)

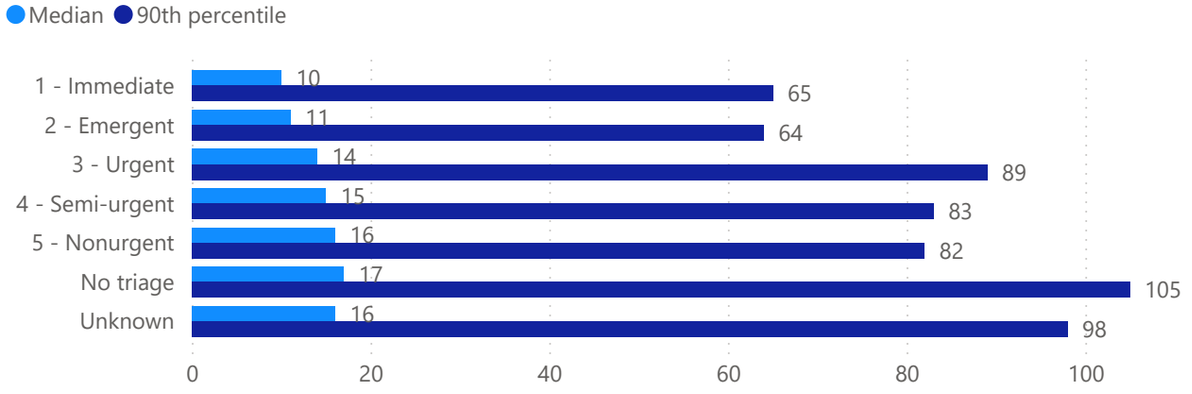


Median boarding time, admitted patients with a time above 0 (minutes)



35–46% of recorded boarding times are 0 minutes, and the share with no time recorded fell from 29% to 21%. This chart shows only patients with a boarding time above 0.

Wait to see a provider, by triage level (minutes)



Recorded waits barely differ by urgency (65 min P90 for Immediate patients), which is implausible for real care. Treat wait times by triage level with caution.

All figures use NHAMCS visit weights. A group is flagged when fewer than 30 sampled visits sit behind it, a simplified version of the NCHS reliability standard, which also uses confidence intervals.

Source: CDC NHAMCS emergency department files, 2018–2022. Weighted national estimates (US).